

Global Challenges in Caregiving for Older Adults: Solutions and Call to Action

There is no doubt that the world's population is experiencing unprecedented longevity. Nurses know only too well that older adults often experience complex and chronic conditions and that there is increased demand for health professionals and informal caregivers to meet their daily needs. In this editorial, we remind readers of the challenges facing caregivers and their care for older adults in a world facing unparalleled changes. We draw readers' attention to the need for wide ranging support and training in caregivers' vital roles, especially that from nurses.

SUPPORT NEEDS FOR CAREGIVERS

The World Health Organization (WHO; 2021) has estimated that by 2030 one in six people worldwide will be aged >60 years, adding to the challenge of what to do about the number of older adults, the unresolved and negative impacts of ageism, and the urgency to provide excellent caring pathways for older adults. With people living longer, old age is often viewed as a disease, something to be dreaded and denigrated; therefore, we need to overcome profound biases against older adults that cause dysfunctional policies to negatively affect their lives, their families, and health care (Aronson, 2019). Too often older adults are undertreated when they are viewed as "elderly" or having multimorbidity. As people age, they often grow to embody stereotypical views

of aging and face discrimination on many levels (Aronson, 2020). Levy and Myer (2004) argue that negative self-perceptions of aging significantly impact health, but older adults having positive self-perceptions are more likely to practice preventive health behaviors. Ageism can place an increasingly high burden on older adults and their caregivers; therefore, strong partnerships are needed between the health sector and caregivers to provide sustainable and meaningful quality care. The life quality of caregivers is dependent on support from other family members, health professionals, and society in general (Pothiban et al., 2020). Health policies need to be supportive and not negate the full participation of older adults in health access and care decisions that encourage them to live healthy lives.

With changes in health care systems, resource and staffing shortages, the coronavirus disease 2019 (COVID-19) pandemic, and climate change effects, the onus of caregiving is moving further onto family members. Health systems are dependent on caregivers keeping their loved ones out of hospitals and needing care and treatment, and the role of nurses is vital in helping caregivers in their roles and providing education and support to them in hospitals as well as community and aged care settings. Despite some views that the COVID-19 pandemic is over, nurses and other health workers continue to witness families under stress as they try to manage car-

ing for their loved ones, having less access to usual care in health systems. Many forms of health care are still not back to pre-pandemic service levels and wait times for treatment and elective surgery remain lengthy.

Alarming, the availability of caregivers is lessening as younger populations are decreasing in many countries. This is a case in point for Japan where population growth has decreased over 2 decades, and the current population is aging rapidly. Caregiving for older adults is often culture-specific, particularly in those societies and cultures where the burden of caring for older family members more often falls on females who are often financially less well-off and have family and domestic responsibilities. In some countries, such as India, caregiving is not confined to children of older adults. Often neighbors and friends feel obligated to assist with caring (Gustafsson et al., 2022). From our own experiences in Thailand, we find that the family takes significant and long-lasting care of older adults within extended families, as enshrined in the caring philosophy of Buddhism. However, family traditions of caring are experiencing significant issues in Thailand, as >20% of the population is now aged ≥65 years, couples are having less children, and more women are entering the workforce.

There is a large body of evidence regarding the deleterious effects that caregiving can place on caregivers. As nurses, we know the multiple mor-

bidities related to aging, including diabetes; cardiovascular, respiratory, and musculoskeletal diseases; and psychological disorders, such as depression and dementia. There needs to be good support, resources, and training for caregivers as their caring burden is multifaceted and affected by social isolation or lack of support from family members and health systems. Caregivers often feel exhausted, have low self-esteem, are chronically stressed, and can be the victims of violence from their care recipients. They can also become perpetrators of abuse in their caregiving roles. Caregivers often feel worn down by the continuous attention that they need to give to older adults, as caregiving has become more intense and of longer duration as older adults live longer with chronic conditions.

Governments and health systems do not provide enough support to families as the numbers of older adults grow, failing to invest in aged care systems and healthy aging, as well as supporting caregivers through funding or respite systems. There are often acute shortages of health care professionals, including nurses, to provide quality safe care to older adults, causing family stress. For example, in Australia, an inquiry (Royal Commission into Aged Care Quality and Safety, 2021) documented failures by the government to provide enough care for older adults living at home or in residential aged care, and not enough RNs and allied health professionals to provide safe and effective care. The Commission highlighted far-reaching recommendations that require significantly more investment in aged care and healthy aging initiatives. In Britain, the government has also been criticized for its lack of investment and reform for older adults within the National Health Service and social care in general (Humphries et al., 2016). Other countries calling on governments to invest more in social and health care of older citizens include Italy (Cepparulo & Giuriato, 2022), South Africa (Kelly et al., 2019), and Thailand (Meemon & Paek, 2019).

The COVID-19 pandemic has significantly affected the lives of older adults. The sociological changes caused by the pandemic have caused an “outbreak of ageism,” and the dominant media portrayal for those aged >70 years has been that they are “helpless, frail, and unable to contribute to society” (Rudnicka et al., 2020, p. 9). Often these vulnerable older adults were the first to become infected and die, and nursing homes became the frontline of mortality. Older adults became isolated as their families were restricted from visiting them at home or in aged care facilities. Caregivers of older adults at home have had significant impacts on their own well-being as they strived to keep their loved ones safe with societies in lockdown and several large waves of viral variants in communities. For example, a study of caregivers of Italian older adults with dementia found that during the pandemic they experienced higher levels of anxiety and depression (Altieri & Santangelo, 2021). However, lack of human connection with family may account for these symptoms, as families and friends were restricted from visiting their loved ones in hospitals and nursing homes. However, a recent study (Silvera et al., 2021) provided evidence that hospitals with fewer visitor restrictions during the pandemic had better outcomes in terms of higher quality of care, patient advocacy, family support, communication, patient safety, and nurse satisfaction. In many settings, social support and health care delivery was, and continues to be, severely restricted for older adults as the pandemic continues into its third year and health systems buckle under the weight of COVID-19.

THE WAY AHEAD AND THE ROLE OF NURSES IN SUPPORTING CAREGIVERS

We are now in the UN Decade of Healthy Ageing (2021–2030), a wide-ranging initiative to reduce health inequities and make improvements to older adults’ lives through collective

action, including primary health and long-term care (WHO, 2021). This initiative’s action areas focus on the development of age-friendly environments; strategies to combat ageism, integrating many levels of care for those who are aging; and good quality long-term care and rehabilitation. Enablers of change for healthy aging need to involve engaging older adults by including their voices for reforms to be meaningful, acceptable, and based in reality; connecting stakeholders; and strengthening research, data, and innovation. Achieving healthy aging goals requires multiple agencies to collaborate, including local and federal governments, non-government agencies, educational institutions, health care sectors, communities, and financial institutions. Primary health care initiatives that educate the public about aging and caring for older adults are vital.

As Fick et al. (2022) described, nurses have a critical role to play in culture change to dismantle ageism within society and to prevent staff shortages in nursing homes. We argue that this role extends into wider communities to help dispel ageism, so that caregiving is valued along with older adults requiring that care. Funding structures in health need to be realigned and bolstered because older adults now require multiple levels of health resources within hospitals, community health centers, and aged care and home care sectors.

Improving caregivers’ skills, knowledge, and coping abilities is critical to ensure that they provide the best care, develop problem-solving skills, and avoid burnout from caregiving. Nurses are in the best position to support caregivers (Wang et al., 2020). When older adults are discharged from the hospital, clinical nurses must assist caregivers to be well-prepared for their roles in home settings and acknowledge their contribution to the well-being of the care recipient. Older adults are often hospitalized for short periods for chronic illnesses and their care at home is thus complex, re-

quiring education, support, and collaboration in care between nurses and caregivers (Hagedoorn et al., 2020). Before discharge, clinical nurses need to carefully train caregivers by providing information and involving them and older adults in decision making. Assessment of caregivers is necessary to help understand how they can contribute to care, and follow-up support is vital if caregivers are to manage and provide quality care to their loved ones. This support can be provided electronically, via phone calls and text messages, or during outpatient appointments. Community nurses are pivotal in supporting and educating caregivers, visiting them and their loved ones at home, as well as providing a point of contact when issues arise. Caregivers need regular interaction with health professionals who can help them manage care challenges as they arise (e.g., clarifying medication regimes and chronic illness care, help navigating technology to get information) (Schurgin et al., 2021). Nurses in hospital and community settings can assist caregivers by providing information on the various resources that are available to them in their local communities or online. Support, understanding, and information exchange is also vital in aged care settings, where family members can contribute meaningfully to the care of older adults. Aged care nurses need to involve family members in decision making and discussions about their involvement in daily care and activities. Thus, nurses' roles are vital in helping caregivers feel supported, contributing to their sense of well-being as well as helping improve the well-being of their care recipient.

Although much research has been performed, we need a better holistic understanding of the needs of older adults across cultures and communities, and possible solutions to ongoing problems, including caregiver distress. In addition, it is urgent that nurses and other health professionals implement a wide range of well-tested innovations that help caregiv-

ers maintain quality of life for older adults, particularly for those caregivers who are at risk due to the intense nature of their roles within the family. Nurses have a vital role in raising issues and proposing policies regarding care of older adults and caregiver support. Nurses can help us gain a better understanding by researching the burden of caregiving and interventions that can be used to support caregivers.

It is also critical that sustained lobbying of policymakers takes place, conducted by collaborations of health care professionals, older adults, and their caregivers to achieve realignment of health care budgets for older adult care. Politicians and policymakers need to be constantly reminded that it is cheaper and more humanitarian to allow individuals to be cared for in their own homes, surrounded by those who can provide individualized attention. Investing in support for family caregivers is therefore much needed. And given that older adults are now being cared for in many health settings, it is vital that nurses and other health professionals are afforded the best training to assist them and caregivers in the community. We need to view older adults as an important resource in society, wise through experience and not diminished by ageist views of weak, feeble people always in need of care.

CONCLUSION

Multi-faceted support and investment for family caregivers is paramount if we are to stop health systems from buckling under the weight of aged care, and to prevent burnout and ill-health affecting caregivers. This is a moral imperative if we value the lives of older adults and their families and care partners.

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Disclosure: The authors have disclosed no potential conflicts of interest, financial or otherwise.

Funding: The writing of this editorial was supported by Visiting Professor funding (S.T.) from the Faculty of Nursing, Chiang Mai University, Thailand.

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doi:10.3928/00989134-20230106-01